

PREOPERATIVE



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- PREPARATION OF PATIENTS FOR SURGERY
- CAN BE USED AS A CURE, PALLIATIVE, PREVENTATIVE, COSMETIC, EXPLORATION AND DIAGNOSIS
- ELECTIVE AND EMERGENCY SURGERY
- AMBULATORY SURGERY (SAME-DAY OR OUTPATIENT SURGERY)

PREOP: INTERVIEW

- ONE OF THE MOST IMPORTANT NURSING ACTION IS THE PREOP INTERVIEW
- CAN BE BEFORE SURGERY OR SAME-DAY AS SURGERY
- PRIMARY PURPOSES
 - OBTAIN PATIENT'S HEALTH HISTORY
 - PROVIDE AND CLARIFY INFORMATION ABOUT PLANNED SURGERY
 - ASSESS PATIENT'S EMOTIONAL STATE AND READINESS FOR SURGERY
 - WHAT ARE PATIENT'S EXPECTATIONS OR SURGICAL OUTCOMES
 - OPPORTUNITY TO ASK QUESTIONS ABOUT SURGERY, ANESTHESIA, AND POSTOP CARE

PREOP: ASSESSMENT

- IDENTIFY RISK FACTORS FOR SURGERY AND ENSURE PATIENT SAFETY
- ESTABLISH BASELINE DATA FOR COMPARISON FOR INTRAOPERATIVE AND POSTOPERATIVE (VITAL SIGNS, RESPIRATORY/CARDIOVASCULAR STATUS, ETC.)
- DETERMINE PATIENT PSYCHOLOGIC STATUS AND PHYSIOLOGICAL FACTORS THAT CAN CAUSE OPERATIVE RISK FACTORS
- IDENTIFY AND DOCUMENT SURGICAL SITE PER AGENCY PROTOCOL
- REVIEW RESULTS OF PREOP TESTS IN THE PATIENT'S RECORD WITH HCP
- IDENTIFY CULTURAL AND ETHNIC FACTORS THAT CAN AFFECT SURGICAL EXPERIENCE
- REVIEW PATIENT'S UNDERSTANDING OF INFORMATION ABOUT SURGERY SO THEY CAN MAKE AN INFORMED CONSENT
- FAMILY HISTORY OF ALLERGIC REACTION TO ANESTHESIA (MALIGNANT HYPERTHERMIA)

PREOP: SUBJECTIVE DATA

- PSYCHOSOCIAL ASSESSMENT (ANXIETY, FEARS, HOPE)
- PAST HEALTH HISTORY ASK ABOUT PREVIOUS HEALTH PROBLEMS OR SURGERIES.
- ALLERGIES
- MEDICATIONS/RECREATIONAL DRUGS
- REVIEW OF SYSTEMS
 - **CARDIOVASCULAR** (HEART FAILURE, MI, HTN, VALVE HEART DISEASE, DEFIBRILLATOR)
 - **RESPIRATORY** (CHRONIC LUNG DISEASE/INFECTIONS, SLEEP APNEA)
 - **NEUROLOGICAL** (ALERTNESS AND ORIENTATION, ABILITY TO FOLLOW COMMANDS)
 - **GENITOURINARY** (CHRONIC KIDNEY DISEASE, UTI, DECREASE IN RENAL FUNCTION)
 - **HEPATIC SYSTEM-** ASSESS FOR LIVER DISEASE, CAN CAUSE INCREASE IN BLEEDING AND METABOLIC ACIDOSIS IF GIVEN LACTATED RINGER'S (LR)

PREOP: SUBJECTIVE DATA

- REVIEW OF SYSTEMS:
 - **INTEGUMENTARY SYSTEM** (SKIN BREAKDOWN, RASHES, PRESSURE ULCERS, TATTOOS, PIERCINGS, ETC.)
 - **MUSCULOSKELETAL SYSTEM** (ARTHRITIS, MOBILITY RESTRICTIONS, NECK INJURIES)
 - **ENDOCRINE SYSTEM** (DIABETES, THYROID DISEASE)
 - **IMMUNE SYSTEM** (IMMUNOCOMPROMISED, CORTICOSTEROIDS)
 - **FLUID AND ELECTROLYTE BALANCE** (VOMITING, DIARRHEA, BOWEL PREPS)
 - **NUTRITIONAL STATUS** (THIN, OBESE, MALNOURISHED)

PREOP: OBJECTIVE DATA

- PHYSICAL ASSESSMENT (H&P AND PHYSICAL EXAM)
- DIAGNOSTIC STUDIES
 - ABGS
 - PULSE OX
 - BLOOD GLUCOSE
 - BUN, CREATININE
 - CHEST X-RAY
 - CBC
 - EKG
 - ELECTROLYTES
 - HCG (PREGNANCY TEST)
 - LIVER FUNCTION TEST
 - COAGULATION BLOOD WORK
 - ALBUMIN LEVEL
 - TYPE AND CROSSMATCH (BLOOD)
 - URINALYSIS

PREOP: TEACHING

- INCREASES PATIENT SATISFACTION, DECREASE FEAR, ANXIETY AND STRESS
- REDUCE POSTOP COMPLICATIONS (DVT, PE, PNEUMONIA, ILEUS, INFECTIONS)
- DECREASES LENGTH OF STAY IN HOSPITAL AND RECOVERY TIME
- PREOP TEACHING USUALLY DONE IN PHYSICIAN/SURGEON'S OFFICE/INPATIENT AND REINFORCED THE DAY OF SURGERY
- PROVIDE WRITTEN INSTRUCTIONS FOR PATIENT AND CAREGIVERS
- DISCHARGE TEACHING REQUIRED FOR ALL PATIENTS
- ASSESS WHAT PATIENT KNOWS AND WANTS TO KNOW AND GIVE PRIORITY TO THOSE CONCERNS
- INCLUDES SENSORY, PROCESS AND PROCEDURAL INFORMATION
- EDUCATION ON DEEP BREATHING, COUGHING AND EARLY AMBULATION UNLESS CONTRAINDICATED
- PAIN MANAGEMENT

PREOP: LEGAL PREPARATION FOR SURGERY

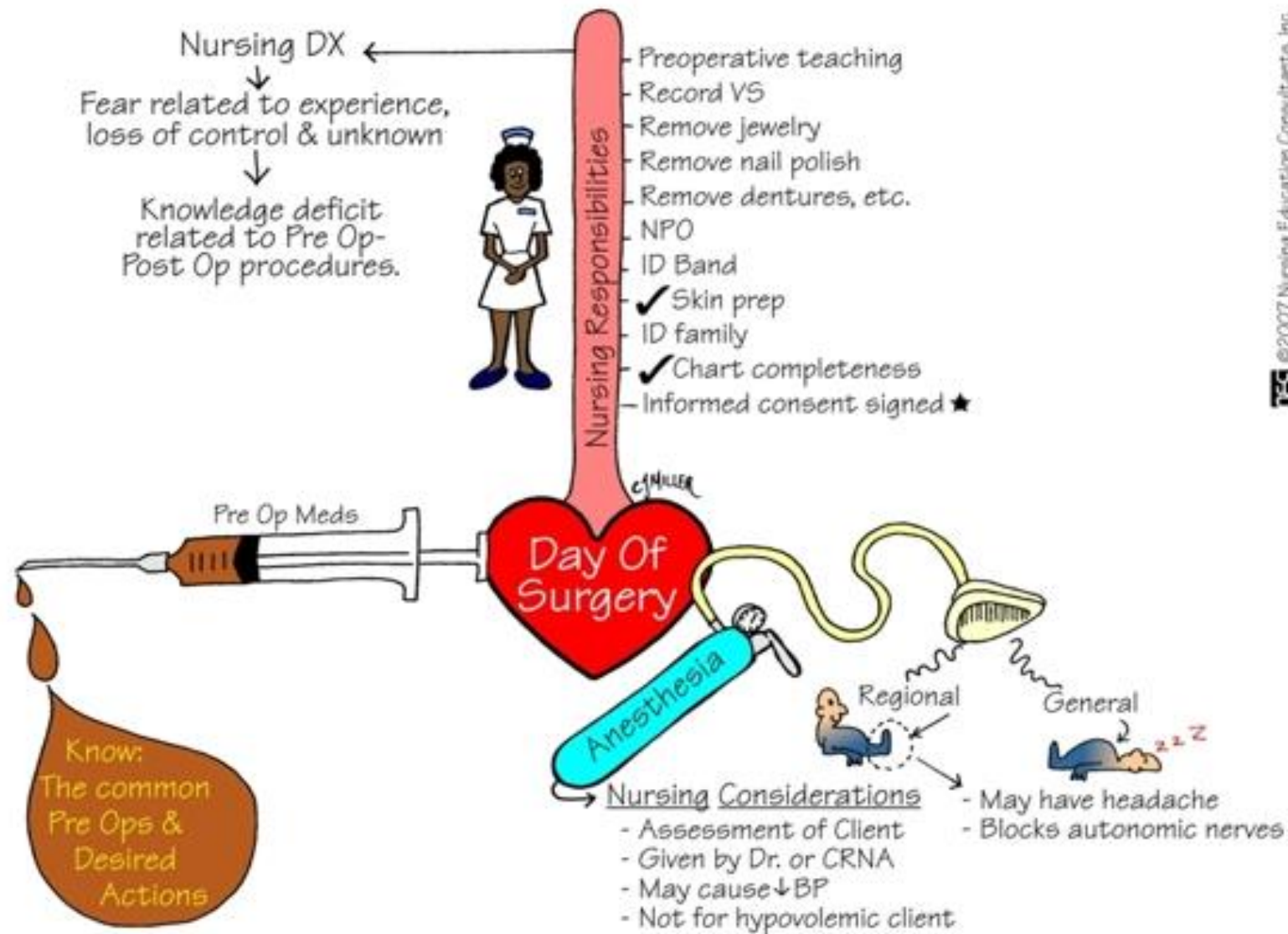
- CHECKING THAT ALL REQUIRED FORMS ARE CORRECTLY SIGNED AND IN PATIENTS CHART (SURGICAL FORM, BLOOD TRANSFUSION, ADVANCE DIRECTIVE, DURABLE POWER OF ATTORNEY)
- PATIENT MUST BE COGNITIVELY COMPETENT AND VOLUNTARILY SIGN INFORMED CONSENT AND MUST BE ADEQUATELY INFORMED ABOUT THE SURGERY/PROCEDURE
- A TRUE MEDICAL EMERGENCY CAN OVERRIDE THE NEED TO OBTAIN CONSENT FROM

PREOP: DAY-OF-SURGERY PREPARATION

- FINAL PREOPERATIVE TEACHING
- ASSESSMENT AND COMMUNICATION OF PERTINENT FINDINGS
- ENSURE PREOP ORDERS ARE DONE AND CHART IS COMPLETE AND TAKEN TO OR
- VERIFY INFORMED CONSENTS, RESULTS OF LAB TEST, DIAGNOSTICS, H&P, CONSULTATIONS, VITAL SIGNS, SKIN PREPARATION, COMPLETED NURSING NOTES
- IDENTIFY, MARK, AND DOCUMENT SURGICAL SITE (SIGNED BY SURGEON AND PATIENT)

PREOP: DAY-OF-SURGERY PREPARATION

- WEAR HOSPITAL GOWN (USUALLY NO UNDERCLOTHES)
- COSMETICS, NAIL POLISH AND ACRYLIC NAILS SHOULD BE REMOVED
- PLACE IDENTIFICATION BAND AND ALLERGY BAND ON PATIENT (ALSO BLOOD BANK BAND, RESTRICTED EXTREMITIES, OSA BAND)
- RETURN ALL PATIENT VALUABLES TO CAREGIVER/FAMILY OR SECURE ACCORDING TO AGENCY POLICY
- REMOVE ANY JEWELRY, GLASSES, CONTACT LENSES, PROTHESIS, DENTURES
- HEARING AIDS SHOULD BE LEFT IN TO ALLOW PATIENT TO BETTER FOLLOW INSTRUCTIONS
- HAVE PATIENT VOID BEFORE PREOP MEDS GIVEN AND BEFORE HEADING TO OR
- GIVE PREOPERATIVE MEDICATIONS
- CAREFULLY CHECK PREOP ORDERS AND CLARIFY THE ORDERS WITH THE SURGEON IF THERE ARE ANY QUESTIONS



PREOP: TRANSPORTING TO OR

- MOVE PATIENT TO STRETCHER TO TRANSPORT TO OR
- CAREGIVERS MAY BE ABLE TO GO WITH PATIENT
- AMBULATORY SERVICE: MAY TRANSPORT PATIENT BY STRETCHER OR WHEELCHAIR
- RECORD METHOD OF TRANSPORT AND WHO TRANSPORTED PATIENT
- PROVIDE HAND-OFF COMMUNICATION TO RN RECEIVING PATIENT
- SHOW CAREGIVER WHERE TO WAIT DURING SURGERY

PREOP: CULTURALLY COMPETENT CARE

- CULTURE OFTEN DETERMINES ONE'S EXPRESSION PAIN, FAMILY EXPECTATIONS AND ABILITY TO VERBALLY EXPRESS NEEDS/PAIN
- CULTURE MAY REQUIRE THAT FAMILY IS REQUIRED IN DECISION MAKING
- RESPECT PATIENT'S DECISIONS
- IF PATIENT DOES NOT SPEAK ENGLISH, IT IS MANDATED THAT THEY HAVE AN QUALIFIED INTERPRETER
- RELIGIOUS BELIEFS MAY REFUSE BLOOD TRANSFUSIONS/PRODUCTS

PREOP: ELDERLY

- AN EVENT THAT HAS LITTLE IMPACT ON A YOUNGER ADULT MAY OVERWHELM AN OLDER ADULT
- HOSPITALIZATION MAY REPRESENT DECLINE IN HEALTH, MOBILITY AND INDEPENDENCE
- OLDER PATIENTS MAY HAVE NEGATIVE PERCEPTIONS OF HOSPITALS
- OLDER ADULTS GREATER RISK FOR COMPLICATIONS AFTER SURGERY AND ADVERSE EFFECTS FROM ANESTHESIA
- OLDER ADULTS CAN HAVE DECREASED ABILITY TO COPE WITH STRESS
- OLDER ADULTS MAY HAVE SENSORY DEFICITS